IGNIS CARE LIMITED
30 Pipit Walk Hemel Hempstead HP3 0FR

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Review Sheet

Last Reviewed 5 Mar 2026 | Last Amended 5 Mar 2026 | This policy will be reviewed as needs require or at the following interval: Annual

Business Impact:	MEDIUM — Changes are important, but urgent implementation is not required, incorporate into your existing workflow.
Reason for this Review:	New Guidance
Changes Made:	Yes
Summary:	This Positive Behaviour Support including Challenging Behaviour Policy and Procedure has been further reviewed adding further personalisation to present information about IGNIS CARE LIMITED, Values, Aims and Objectives. For these changes to reflect in the policy the system details questionnaire will need to be updated.
Relevant Legislation:	• Autism Act 2009 • The Care Act 2014 • Equality Act 2010 • The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014 • Health and Safety at Work etc. Act 1974 • Human Rights Act 1998 • Management of Health and Safety at Work Regulations 1999 • Mental Capacity Act 2005 • Mental Capacity Act Code of Practice • Safeguarding Vulnerable Groups Act 2006 • Data Protection Act 2018 • Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR) • UK GDPR • Data (Use and Access) Act 2025
Underpinning Knowledge:	• Author: NICE, (2015), NG11 - Challenging behaviour and learning disabilities: prevention and interventions for people with learning disabilities whose behaviour challenges [Online] Available from: https://www.nice.org.uk/guidance/ng11/chapter/recommendations [Accessed: 05/03/2026] • Author: CQC, (2025), Regulations for service providers and managers [Online] Available from: https://www.cqc.org.uk/guidance-regulation/providers/regulations-service-providers-and-managers [Accessed: 05/03/2026] • Author: GOV UK, (2025), Oliver McGowan code of practice [Online] Available from: https://www.gov.uk/government/publications/oliver-mcgowan-code-of-practice [Accessed: 05/03/2026] • Author: Hastings, R P, Allen, D, Baker, P, Gore, N J, Hughes, J C, McGill, P, Noone S J and Toogood, S, (2013), A conceptual framework for understanding why challenging behaviours occur in people with developmental disabilities [Online] Available from: https://www.researchgate.net/publication/263527512_A_conceptual_framework_for_understanding_why_challenging_behaviours_occur_in_people_with_developmental_disabilities [Accessed: 05/03/2026] • Author: BILD, (2024), Positive Behaviour Support (PBS) [Online] Available from: https://www.bild.org.uk/positive-behaviour-support-pbs/ [Accessed: 05/03/2026] • Author: GOV.UK, (2019), Reducing the need for restraint and restrictive intervention [Online] Available from: https://www.gov.uk/government/publications/reducing-the-need-for-restraint-and-restrictive-intervention [Accessed: 05/03/2026] • Author: National Institute of Health and Care Excellence (NICE), (2015), Violence and aggression: short-term management in mental health, health and community settings [Online] Available from: https://www.nice.org.uk/guidance/ng10 [Accessed: 05/03/2026] • Author: The Restraint Reduction Network, (2025), RRN Training Standards [Online] Available from: https://restraintreductionnetwork.org/training-standards/ [Accessed: 05/03/2026] • Author: CQC, (2025), Services for autistic people and people with a learning disability [Online] Available from: https://www.cqc.org.uk/guidance-providers/autistic-people-learning-disability/right-support-right-care-right-culture [Accessed: 05/03/2026] • Author: CQC, (2018), Brief guide: Positive behaviour support for people with behaviours that challenge [Online] Available from: https://www.plymouthonlinedirectory.org/media/1769/CQC-Brief-Guide-to-PBS/pdf/CQC_brief_guide_to_PBS_for_people_with_behaviours_that_challenge_v4.pdf [Accessed: 05/03/2026] • Author: Skills for Care, (2024), Supporting people with challenging or distressed behavior [Online] Available from: https://www.skillsforcare.org.uk/Developing-your-workforce/Care-topics/Supporting-people-with-challenging-or-distressed-behaviour/Supporting-people-with-challenging-or-distressed-behaviour.aspx [Accessed: 05/03/2026] • Author: Department of Health, (2014), Positive and Proactive Care: reducing the need for restrictive interventions [Online] Available from:

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Document Monitor

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	https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/300293/JRA_DoH_Guidance_on_RP_web_accessible.pdf [Accessed: 05/03/2026]
Suggested Action:	• Encourage sharing the policy through the use of the QCS App
Equality Impact Assessment:	QCS have undertaken an equality analysis during the review of this policy. This statement is a written record that demonstrates that we have shown due regard to the need to eliminate unlawful discrimination, advance equality of opportunity and foster good relations with respect to the characteristics protected by equality law.

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1. Purpose

1.1 About IGNIS CARE LIMITED

At Ignis Care, we aim to provide safe, stable, and person-centred support for people across a range of needs, enabling them to live fulfilling lives in the community.

Ignis Care delivers support for adults and children with **mental health conditions, learning disabilities and autism, dementia, sensory and physical disabilities**, and other complex needs. We work within **supported living and domiciliary care pathways**, always ensuring tenancy separation in line with the **REACH framework** and the Real Tenancy Test.

Our approach is rooted in **trauma-informed, values-based care**, with a focus on independence, recovery, and preventing crisis escalation. We employ skilled staff who are trained in Positive Behaviour Support (PBS), safeguarding, and specialist interventions, ensuring people are supported safely and compassionately.

Our ambition is to empower each person to achieve their personal goals — whether that means developing daily living skills, sustaining a tenancy, rebuilding relationships, returning to education or employment, or living safely with dignity in later life. We do this through tailored care planning, strong clinical governance, and close collaboration with families, commissioners, housing partners, and NHS/local authority professionals.

What are the values of IGNIS CARE LIMITED

At Ignis Care, our values guide every decision, every shift, and every interaction. We are committed to:

- **Safety and Skilled Support** – providing high-quality, evidence-based care and support so that every person can achieve their optimum health, wellbeing, and independence.
- **Respect and Dignity** – treating every individual we support, every colleague, and every visitor with respect at all times, and recognising the uniqueness of each person.
- **Human Rights and Citizenship** – upholding the rights of every person to be included, to belong, and to have their voice heard in decisions that affect their life.
- **Choice and Independence** – encouraging informed choices and supporting decision-making, recognising that independence looks different for each person.
- **Privacy and Trust** – safeguarding personal information and respecting privacy in all aspects of daily life.
- **Personal Fulfilment** – enabling people to pursue meaningful goals, activities and relationships that give purpose and joy.
- **Continuous Learning and Improvement** – fostering a culture of reflection, training and feedback so that our staff grow and our service evolves.
- **Trauma-Informed, Person-Centred Care** – understanding people's stories and experiences, and shaping support that responds with compassion, patience and hope.

What are the Aims and Objectives of IGNIS CARE LIMITED

1.2 Meeting Regulation 9: Person-centred care

This policy emphasises the importance of delivering person-centred care, ensuring that all services are tailored to the individual needs, preferences, and values of each Service User. This approach promotes dignity, respect, and independence, enhancing the overall quality of life for Service Users.

1.3 Meeting Regulation 10: Dignity and respect

This policy ensures that all Service Users are treated with dignity and respect at all times, promoting a culture of kindness and compassion. Staff are trained to recognise and uphold the intrinsic value of each individual, promoting an environment where Service Users feel valued and respected.

1.4 Meeting Regulation 11: Need for consent

This policy ensures that valid consent is obtained from Service Users for all aspects of their care, support and treatment, in accordance with legal and ethical standards, including the Mental Capacity Act 2005. Informed consent procedures are in place to ensure that Service Users understand and agree to their Care Plans, respecting their autonomy and rights. This policy also ensures that when Service Users lack the capacity to consent, decisions are made in their best interests, following the guidelines set out by the Mental Capacity Act.

1.5 Meeting Regulation 12: Safe care and treatment

This policy is designed to ensure that all care, support and treatment provided is safe, effective, and in line with best practices and regulatory standards. Risk assessments, staff training, and continuous monitoring are implemented to maintain the highest levels of safety and quality in care and support delivery.

1.6 Meeting Regulation 17: Good governance

This policy ensures the implementation of strong governance practices, including continuous quality improvement, risk management, and compliance with all regulatory requirements. Effective leadership and accountability structures are in place to support high standards of care/support and service delivery.

1.7 Meeting Regulation 18: Staffing

This policy ensures that staffing levels and competencies are sufficient to meet the needs of Service Users, providing high-quality care and support. Ongoing training, professional development, and performance evaluations are conducted to maintain a skilled and capable workforce.

1.8 To ensure that a range of complex needs are fully assessed in order to understand why Service Users present with stressed, distressed behaviours or reactions. As well as meeting the Service User's needs, the policy also aims to ensure that they are supported

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in a personalised way that enhances their quality of life.

1.9 This policy will refer to 'a behaviour' of the Service User. In this policy we mean any behaviour or reaction of Service Users that other people may consider to be challenging or concerning.

1.10 To demonstrate that all staff are aware of Positive Behaviour Support (PBS) Care Plans which are in place, the need for Care Plans to be developed if required and the ongoing review of the effectiveness of such Care Plans.

1.11 To ensure that Service Users are aware of what to expect in the promotion and development of Positive Behaviour Support (PBS) Care Plans within IGNIS CARE LIMITED.

1.12 To ensure that all staff receive appropriate training and support to meet the needs of Service Users, including PBS and its implementation.

This also includes where required the Oliver McGowan training.

1.13

Key Question	Quality Statements
CARING	QSC1: Kindness, compassion and dignity
CARING	QSC2: Treating people as individuals
EFFECTIVE	QSE1: Assessing needs QSE2: Delivering evidence-based care & treatment
EFFECTIVE	QSE2: Delivering evidence-based care & treatment QSE3: How staff, teams & services work together
RESPONSIVE	QSR1: Person-centred care
SAFE	QSS4: Involving people to manage risks QSS5: Safe environments
WELL-LED	QSW1: Shared direction and culture QSW2: Capable, compassionate and inclusive leaders

1.14 Relevant Legislation
- Autism Act 2009
- The Care Act 2014
- Equality Act 2010
- The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014
- Health and Safety at Work etc. Act 1974
- Human Rights Act 1998
- Management of Health and Safety at Work Regulations 1999
- Mental Capacity Act 2005
- Mental Capacity Act Code of Practice
- Safeguarding Vulnerable Groups Act 2006
- Data Protection Act 2018
- Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR)
- UK GDPR
- Data (Use and Access) Act 2025

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2. Scope

2.1 Roles Affected:
- All Staff

2.2 People Affected:
- Service Users

2.3 Stakeholders Affected:
- Family
- Advocates
- Representatives
- Commissioners
- External health professionals

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- Local Authority

3. Objectives

3.1 To enable Service Users to lead fulfilling and socially inclusive lives regardless of the concerns posed by their behaviour.

3.2 To enable staff to gain an understanding of the needs of Service User's behaviours. With greater understanding, staff can identify potential triggers, provide effective support and reduce the risk of escalation or further distress.

3.3 To take a positive and proactive approach to the management of behaviour based on the principles of Positive Behavioural Support (PBS).

3.4 To carefully assess and sensitively manage presenting and potential risks whilst also acknowledging that positive risk taking is essential to the personal growth and development of Service Users.

3.5 To ensure staff develop knowledge and skills that enable them to act positively to behaviour, reducing the likelihood of increased behaviour incidents.

3.6 To outline the governance responsibilities of IGNIS CARE LIMITED in ensuring the appropriate information, support and training is provided to enable staff to identify key areas of need which, if left unmet, may give rise to behaviour in Service Users.

3.7 Service Users and prospective Service Users will be aware of the standard and practices of PBS at IGNIS CARE LIMITED.

4. Policy

4.1 IGNIS CARE LIMITED will follow the guidance from the Department of Health, 'Positive and Proactive Care: reducing the need for restrictive interventions (2014)'. This states that services that support Service Users whose needs and histories mean they are likely to present with behaviours that challenge, should use recovery-based approaches and delivery of care in accordance with the principles of positive behavioural support.

This applies to services that work with Service Users who present with a behaviour, as well as services for Service Users who are elderly, confused, who may become agitated and may be living with dementia.

4.2 IGNIS CARE LIMITED will have resources and knowledgeable, competent staff available to respond appropriately to behaviours as they present. This includes being aware of the environment and how this can minimise triggers that may escalate those behaviours, whilst complying with health and safety and other policies associated with safeguarding Service Users, staff and visitors.

Staff will maintain an open and honest approach towards all Service Users at all times and deliver Support in a consistent and non-judgemental manner.

4.3 Positive Behaviour Support (PBS) is a person-centred framework for providing long-term support to people with a learning disability, and/or autism, including those with dementia and mental health conditions, who have or may be at risk of developing behaviours. It is a blend of person-centred values and behavioural science and uses evidence to inform decision making.

Behaviours usually happen for a reason and may be the person's only way of communicating an unmet need. PBS helps us to understand the reason for the behaviour so we can better meet people's needs, enhance their quality of life and reduce the likelihood that the behaviour will happen.

All staff at IGNIS CARE LIMITED recognise that these behaviours often have multiple causative factors.

4.4 PBS Competence Framework

The UK PBS competence framework provides a detailed framework of the things you need to know and need to do when delivering best practice PBS to Service Users.

The objectives are:

- Service Users will benefit from high-quality, evidence-based support delivered by competent professionals working as part of a multidisciplinary team
- Staff will benefit from professional development and occupational standards
- IGNIS CARE LIMITED will be able to employ staff with a greater degree of certainty about competence and quality
- Commissioners of services at IGNIS CARE LIMITED will have a greater understanding of the nature of PBS in practice
- Practice-based research will contribute to the growing evidence base of PBS

4.5 Key Components of PBS (Source: Positive Behavioural Support (PBS) in the UK: A State of the Nation Report, 2022)

Rights and Values: A focus on rights and good lives

- Person-centred foundation
- Constructional approaches and self-determination
- Partnership working and support for key people
- Elimination of aversive, restrictive and abusive practices

Theory and Evidence Base: Ways to understand behaviour, needs, and experience

- A biopsychosocial model of behaviours that challenge
- Behavioural approaches to learning, experience and interaction
- Multi-professional and cross-disciplinary approaches

Process and strategy: A systematic approach to high quality support

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- Evidence-informed decisions
- High-quality care and support environments
- Bespoke assessment
- Multi-component, personalised support plans
- Implementation, monitoring, and evaluation

4.6 This policy details the steps to be taken to assess, plan, implement and evaluate Service Users' PBS Care Plans where required.

4.7 PBS puts the Service User first and helps staff at IGNIS CARE LIMITED understand and address the underlying needs that leads to individual Service User behaviours. The focus is on better supporting the Service User through improving their quality of life.

PBS is proactive, meaning it focuses on getting the environment right for the individual, rather than just responding to the presenting behaviour.

4.8 PBS does not use force and at no point will the Service User be punished for displaying behaviours.

4.9 To support and meet the needs of Service Users IGNIS CARE LIMITED will ensure that all relevant staff receive training, education, support in PBS, The Oliver McGowan Training (where applicable) and any local and service specific training in line with the training plan at IGNIS CARE LIMITED.

4.10 Policy Accessibility

IGNIS CARE LIMITED understands that some Service Users may take in and retain information in different ways. To support full understanding and engagement, this policy is available in accessible formats. The ReciteMe tool has various ways of making this policy accessible by providing this policy in:

- Audio
- Large print
- Multiple languages

This policy can also be made available in:

- Easy-read versions
- Single-policy view to reduce navigation and complexity

These options are in place to help Service Users to understand and engage with this policy more easily.

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5. Procedure

5.1 The lead for this policy and procedure covering Positive Behaviour Support at IGNIS CARE LIMITED is Registered Manager: Mrs Miriam Chengeto Cummings.

It is the responsibility of this lead to ensure this policy remains up to date and in line with practices carried out at IGNIS CARE LIMITED.

5.2 Safeguarding and Risk Management

Purpose

The Registered Manager, Mrs Miriam Chengeto Cummings, manages how risks are identified, assessed, and reviewed to ensure the safety, wellbeing, and rights of Service Users supported under Positive Behaviour Support (PBS). It ensures compliance with safeguarding duties and promotes a proactive approach to risk management.

Identifying Risks

Risks are identified through a range of ongoing and structured processes, including:

- Initial and ongoing Service User assessments (including behavioural, environmental, and health assessments)
- Service User PBS assessments and functional behaviour analysis
- Service User risk assessments and behaviour support plans
- Incident reports, near-miss reporting, and safeguarding concerns
- Feedback, complaints, and concerns raised by Service Users, families, staff, or professionals
- Multi-disciplinary reviews and professional input

All identified risks are documented, reviewed, and communicated appropriately.

Managing and Reducing Risks

Risks are managed using a proactive and least restrictive approach, in line with PBS principles. This includes:

- Developing and implementing person-centred Service User PBS plans that focus on prevention, skill development, and positive strategies
- Implementing control measures identified through risk assessments
- Ensuring staff are trained and competent to follow PBS plans and safeguarding procedures
- Using restrictive practices only as a last resort, in line with legislation and IGNIS CARE LIMITED policy
- Regularly reviewing risk management strategies to ensure they remain effective and proportionate

Safeguarding

Any concern relating to abuse, neglect, or harm is managed in accordance with the Safeguarding Adults Policy and Procedure at IGNIS CARE LIMITED. This includes:

- Immediate action to protect the Service User from harm

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- Reporting safeguarding concerns promptly to the appropriate internal lead and external authorities where required
- Maintaining accurate records of safeguarding concerns, actions taken, and outcomes
- Ensuring Service Users are supported and listened to throughout the safeguarding process

All staff are required to follow safeguarding procedures and raise concerns without delay.

Complaints and Concerns

Complaints, concerns, and feedback are recognised as an important part of risk identification and quality improvement. All complaints are managed in line with the Complaints, Suggestions and Compliments Policy and Procedure of IGNIS CARE LIMITED, which ensures that:

- Complaints can be raised safely and without fear of reprisal
- Concerns are investigated promptly, fairly, and transparently
- Outcomes and learning are used to improve practice and reduce future risks
- Complainants are informed of the outcome and their right to escalate concerns

Responsibility

All staff are responsible for identifying and reporting risks and safeguarding concerns. The Registered Manager, Mrs Miriam Chengeto Cummings, is responsible for ensuring appropriate risk management systems are in place and that safeguarding and complaints procedures are followed.

5.3 A behaviour can pose a risk to the Service User or others around them. These behaviours are not a diagnosis, but some people living with learning disabilities, autism, dementia and mental health conditions are more likely to display behaviour towards themselves, carers, family members and other people. Behaviours can be displayed by anyone, regardless of any condition or diagnosis.

Some behaviours that may be presented are:

- Aggression (verbal or physical)
- Self-harm (head banging, pulling their own hair out)
- Sexually inappropriate behaviour
  - Undressing in public
  - Fondling genitals
  - Touching someone inappropriately
- Pica (eating objects that are not fit for human consumption)
- Destructiveness
- Disruptiveness
- Asking the same question or doing the same activity over and over again
- Restlessness
  - Pacing up and down
  - Walking with purpose
  - Fidgeting
- Night-time waking and sleep disturbance
- Faecal smearing

IGNIS CARE LIMITED acknowledges that no policy or procedure can eliminate a behaviour and there is no 'one size fits all' approach to its management. However, the following procedures are based fully on evidenced-based practice, underpinned by person-centred Support planning and the recognition that everyone is unique and individual.

Where emotional need cannot be met, skilled management can often divert or distract from behaviour presented by Service Users. Staff can refer to the 'Stop and Pause' document (in the Forms section of this policy) to understand why Service Users may present with a behaviour and to establish techniques to support effective person-centred Support planning.

5.4 IGNIS CARE LIMITED acknowledges that there may be circumstances where a behaviour presents in Service Users some time after service has commenced. This can often arise when providing Support for Service Users living with dementia, whose condition may have progressed.

IGNIS CARE LIMITED acknowledges that these incidents must be recorded and a suitable assessment and Care Plan formulated. IGNIS CARE LIMITED will seek advice and support from other professionals where necessary, and will ensure that staff who are trained and confident attend to provide Support to the Service User.

5.5 Assessment of Behaviours

Positive Behaviour Support (PBS) provides a values-based framework for the needs and risk assessment of developing an understanding of Service User's behaviour and to assist with the planning of effective support.

Staff must ensure that assessments of Service Users include consideration of the following:

- Their abilities and needs (in particular, their expressive communication and receptive communication)
- Any physical or mental health problems and the effect of medication, including side effects
- Any learning disabilities (including the severity of the learning disability and the presence of autism or other behaviours associated with a learning disability and/or autism)
- Types of dementia
- Response to any previous interventions for a behaviour
- The impact of the behaviour on the Service User's quality of life and that of their family members or carers; independent living skills and educational or occupational abilities; social and interpersonal history, including relationships with family members, carers, staff (or other people with a learning disability or dementia such as those the Service User lives with)
- Aspects of the Service User's culture that could be relevant to their behaviour

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- Their life history, including any history of trauma or abuse
- Recent life events and changes to routine
- The Service User's sensory profile, preferences and needs
- The physical environment, including heat, light, noise and smell
- The home environment, including the range of activities available, how it engages people and promotes choice, and how well structured it is

The functional assessment should consider the Service User's:

- History
- Physical health (including pain assessment)
- Mental health
- Communication and social skills
- The broader social environment
- What happens immediately before the behaviour starts (antecedents) and what happens as a result of it (consequences)

All staff must recognise that challenging, concerning, stressed and distressed behaviours often have multiple causative factors.

The functional assessment should also consider

- A clear description of the behaviour (including classes or sequences of a behaviour that occur together)
- Identification of the events, times, and situations that predict when the behaviour will and will not occur across the Service User's full range of typical daily routines and environment
- Identification of the consequences that maintain the behaviour (that is, the purposes or functions that the behaviour appears to serve for the Service User)
- The development of one or more summary statements or hypotheses that describe the relationship between personal and environmental triggers, the behaviour and its reinforcers
- The collection of direct observational data that support the summary statements

Assessment should include a baseline measure of current behaviour, its frequency and intensity, and repeated measurements in order to evaluate change.

Assessment should be a dynamic process and repeat assessments should always follow any change in presentation of the Service User's behaviour.

Assessment of more complex behaviours should always be multidisciplinary, and whenever possible, involve the Service User.

5.6 Risk Assessment and Management

The assessment of risk is a key process in the identification of hazards and factors that lead to or contribute to challenging, concerning, stressed and distressed behaviours. The risk assessment will assist in the identification of preventative measures to reduce the risk and impact of potential behaviours.

5.7 Positive Behaviour Care Plans

This focuses on providing a detailed and personalised description of how best to support each Service User with a behaviour including those with developmental disabilities, including prompts to guide the behaviour of those supporting them, strategies to redesign their environment and therefore reduce potential behaviours, and a plan to develop their skills and appropriate behaviour; to deliver support in a way that is consistent with the Behavioural Support Plan (BSP).

Best Practice Standards should include:

- Staff at IGNIS CARE LIMITED understand the rationale of a BSP and its uses, and are able to implement it
- An overview of the Service User's skills and needs; staff contribute to the assessment process as part of a multidisciplinary team
- Constructing a model that explains the functions of the Service User's behaviour and how those are maintained, by identifying the environmental variables associated with the presenting behaviour for the Service User
- Devising and implementing multi-element evidenced-based support strategies based on the overview and model by:
    - Staff identifying antecedent strategies, and demonstrating they are understood and followed
    - Staff understanding teaching strategies and protocols within the Service User's PBS
    - Staff teaching and supporting, or increasing development of a new skill / communication based on the PBS
    - Staff increasing Service User's engagement levels via strategies outlined
    - Demonstrating implementation of strategies related to the Service User's plan, such as changes to the physical environment, supporting health needs
    - Offering choice and promoting independence
- Devising and implementing a least restrictive crisis management strategy in response to incidents of behaviour that are not preventable, which aim to minimise escalation and reduce the risk of harm to the Service User and others, by:
    - Identifying early warning signs that challenging, concerning, stressed and distressed behaviours may occur
    - Remaining calm and implementing the crisis plan quickly
    - Having detailed records that are kept and used to learn from incidents and reviewing the BSP
- Developing the plan; outlining responsibilities and time frames
- Monitoring the delivery of the BSP
- Evaluating the effectiveness of the BSP
- The BSP as a live document
- **Primary Preventative Strategies:** Designed to enhance the Service User's quality of life, meet key needs associated with the behaviour(s), avoid necessary exposure to the behaviour, develop the Service User's skills and competencies to cope better with

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any exposure to predictor

- **Secondary Preventative Strategies:** Implemented in response to early indicators of mounting anxiety and agitation. These aim to prevent a further escalation, not to teach the Service User. Strategies typically involve approaches such as: providing extra attention and assistance, injecting humour, prompting engagement in relaxing activities, distraction and diverting to preferred activities
- **Reactive Strategies:** Record the agreed, planned strategies to be used in response to an acute crisis. They may include the continued use of de-escalation approaches, evacuating the area and, in exceptional circumstances, physical intervention and/or the administration of 'as required' medication. In the case of physical interventions and the use of 'as required' medication, this must be agreed as part of a wider multidisciplinary decision and be in line with the Mental Capacity Act (2005)

PBS plans will often make reference to additional Health Promotion Care Plans or Health Action Plans for some mental health conditions or physical conditions; particularly where there is pain, which may influence challenging, concerning, stressed and distressed behaviours. In this case, actions which aim to meet health needs may be important primary preventative strategies. Additionally, all Service Users will have additional Care Plans which detail what is important to them and all meaningful activities. Person-centred goals and preferences will be taken into full account within the PBS Care Plan.

5.8 Effective Communication

There is a strong link between communication difficulties and behaviours that might be of risk or that can have a significant impact. The greater the communication difficulty, the greater the likelihood that the Service User will show a behaviour.

The reasons for this may be:

Understanding – the ability to understand what they hear and see.

- If the Service User does not understand what they are being asked to do, it could incorrectly be assumed they have understood, so when they do not do what they are asked, it could be assumed they are being difficult
- If the Service User does not understand what is happening they may be frightened, anxious or confused, which can lead to a behaviour
- Service Users may find it difficult to make transitions and this can be even more difficult if they do not know what is happening
- Communication and visual aids must be at the right level (for example, a symbol timetable is only helpful if the Service User understands symbols)
- Understanding is a hidden disability. It can be overestimated what people can understand and if sentences are used that are too long and complicated

Check if the Service User has understood:

- By monitoring their expression – the ability to express what they want/feel/think
- The Service User might have words, but the more anxious they are, the harder it is to make themselves understood
- If the Service User does not have words, then they may try to find other ways of letting others know what they want
- If the Service User is unable to have their needs met, they may become frustrated, possibly leading to a behaviour, or they may give up and become passive and withdrawn

Reassurance, Support and Attention

Some Service Users may find it difficult to get someone's attention, and this can show in their behaviour. It is really important that the following is considered:

- Is communication supported at the right level?
- Has every effort been made to give the Service User the tools and resources to express what they need?
- If the communication is complex, does the Service User need to be referred to a speech and language therapist for some extra help?

The more the Service User is able to meet their own needs through communicating, the less they will need to rely on behaviour, therefore it is important to plan and have a structure in place.

Please also refer to the Accessible Information Standard (AIS) Policy and Procedure.

Best Practice Using Active Listening

Active listening is a key skill in effective communication, particularly when working with Service Users who may have communication difficulties. Active listening involves fully concentrating, understanding, responding, and remembering what the Service User is communicating. Staff at IGNIS CARE LIMITED should use the following examples of best practices using active listening:

Show That You Are Listening:

- Non-verbal Cues: Use eye contact, nodding, and open body posture to show that you are engaged and paying attention
- Verbal Acknowledgements: Use phrases like "I see," "Go on," or "I understand" to encourage the Service User to continue speaking

Reflecting and Paraphrasing:

- Reflect Back: Repeat or paraphrase what the Service User has said to show understanding. For example, "It sounds like you are feeling upset because you don't understand what's happening. Is that right?"
- Clarification: Ask clarifying questions to ensure you have understood correctly. For example, "When you say you feel lost, do you mean you're unsure about what to do next?"

Empathise:

- Acknowledge Feelings: Recognise and validate the Service User's emotions. For example, "I can see that this is very frustrating for you."
- Use Empathetic Statements: Use statements that show you understand their feelings. For example, "It must be really hard to feel confused and not know what's going on."

Summarise

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- Provide a Summary: Summarise what the Service User has said to ensure understanding and show that you have been listening. For example, "So, you're feeling anxious because you don't know what's going to happen next, and you would like some more information about your afternoon activities."

Avoid Interrupting:

- Give Space to Speak: Allow the Service User to express themselves fully without interrupting them. This shows respect and patience, which can help reduce anxiety and frustration
- Wait for Natural Pauses: Use natural pauses in the conversation to ask questions or provide responses

Ask Open-Ended Questions:

- Encourage Dialogue: Use questions that require more than a yes or no answer to encourage the Service User to express themselves more fully. For example, "Can you tell me more about what makes you feel anxious?"
- Explore Feelings and Thoughts: Ask questions that help explore their feelings and thoughts. For example, "What do you think would help you feel more comfortable?"

Use Silence Effectively:

- Allow Time to Think: Give the Service User time to think and process their thoughts before responding. Silence can be a powerful tool to give them space to gather their thoughts
- Be Patient: Show patience and understanding that some Service Users may need more time to express themselves

Respond Appropriately:

- Tailor Your Responses: Ensure your responses are appropriate to the Service User's needs and communication style. Be mindful of your tone, language, and body language
- Provide Reassurance: Offer reassurance and support, especially if the Service User is expressing concerns or fears. For example, "I'm here to help you, and we will figure this out together."

By incorporating these active listening practices alongside Care Plans, staff at IGNIS CARE LIMITED can improve communication with Service Users, better understand their needs and concerns, and provide more effective and empathetic support.

5.9 Person-Centred Active Support

If Service Users are engaged in meaningful things, has interests, has good social relationships and develops the skills to help them get what they need, they are less likely to engage in behaviours that make life difficult for them and others. Active support provides a good basis for enhancing Service User's quality of life by increasing opportunities for engagement and spending time with others in the community.

There is no expectation that Service Users will become more skilled or independent as a result of active support. However, active support is a lifestyle - not an 'active treatment' or skills teaching programme. Active support from IGNIS CARE LIMITED will follow these key components:

- Where appropriate to do so Support staff will support the use of the PAL assessment tool (A copy can be found within the QCS Resource Centre).
- The Service User will be offered opportunities to take part in everyday activities that are meaningful to them. This may be work, leisure or education. Identifying activities that are meaningful to them supports personal routines but can also be responsive and spontaneous
- All staff will work closely with the Service User to ensure success and take account of their personal preferences which will link to their person-centred plan or the Service User's own goals
- Staff will monitor, record and report on levels of engagement within the Service User's Care Plan
- Staff will review any behaviour incidents to ensure that lessons are learned and that any alternatives to any restrictive practices are sought

5.10 Creating High-Quality Care and Support

To ensure that IGNIS CARE LIMITED and those supporting the Service User operate from a best practice person-centred foundation to enable a high quality of life for all concerned. This includes mitigating risk factors for the development and maintenance of a behaviour, and creating high-quality and supportive environments. The likelihood and impact of a behaviour is likely to be reduced in environments that meet the Service User's social, physical and mental health needs, and that successfully facilitate engagement, communication and choice making.

The following best practice standards should be in place:

Ensure IGNIS CARE LIMITED is Values Led - The key focus of Support is enablement:

- Show dignity, respect, warmth, empathy and compassion in all interactions
- Treat every Service User as a person and provide support that is tailored to meet their need
- Arrange and support participation in community activities and events
- Search out and support the development of relationships
- Arrange and support participation in activities of everyday life
- Arrange and support meaningful choice
- Arrange and support opportunities for learning and development
- Help and support behaviour and daily interactions that make the Service User look and feel good
- Minimise any restriction of activities or movement; and use positive handling strategies when needed in emergency situations
- Ensure staff at IGNIS CARE LIMITED know the Service User

Building a Good Rapport

Building rapport, or connection, has real impact. Service Users often respond more to some staff than others. The Service User may show this by smiling, laughing and communicating more when they are with them.

Service Users may be less likely to show a behaviour, when they are being supported by someone with whom they have a good rapport – they might be more willing to engage and interact.

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Staff should:

- Find out what they have in common (whether it is activities you enjoy or things they like to eat)
- Pay attention to how they communicate so that they are not asking too much of the person or being too bossy
- Notice and comment when the Service User says or does things – show interest
- Make sure the Service User is included in decisions and conversations
- Listen to the subtle ways they say what they want or do not want (even if it is just by turning their head away)
- Try to understand what those subtle signs mean. Be patient and allow time for the Service User to get their message across. Once staff understand what the person means, let them know it is understood ("Oh, you want a biscuit!")
- Be responsive – try to provide what the Service User is asking for, even if it is in small amounts
- Build trust. Do not offer things that cannot be delivered. Be true to your word
- Consider how to give the Service User bad news, and prepare to do this sensitively, clearly and kindly
- Identify and describe how the Service User expresses enjoyment and displeasure in activities
- Directly support the Service User to access things that are important to them (preferences) and balance this with the things that are required for them to have a good quality of life
- Support the Service User across a range of activities and contexts
- Reflect on relationship with the Service User

Match Support with the Service User's Capabilities, Goals and Outcomes that are Important to Them:

- Collect information about Service User's strengths, needs, preferences, hopes, dreams and desires
- Help schedule the implementation of personally important goals
- Help measure progress towards personally important goals

Establish Clear Roles and Effective Teamwork at IGNIS CARE LIMITED:

- Demonstrate appropriate levels of support to the Service User, rather than doing too much for them, or not engaging with them
- Demonstrate the difference between Support and providing personalised and active support
- Key workers are allocated for Service Users
- Provide peer support to colleagues
- Actively participate in teamwork; attend and participate in team meetings and supervision
- Reflect on own actions and feelings, and how these impact on the actions and feelings of others
- Seek support from Mrs Miriam Chengeto Cummings, supervisor/peers when needed
- Declare any personal and/or professional relationships that will or might impact on the job role or organisation's functioning
- Attend to own physical, psychological and emotional wellbeing

Support Communication By:

- Effectively communicating and supporting the use of core communication systems (e.g. non-verbal, verbal, gestural, pictorial/textual) in all interactions
- Using appropriate communication with different Service Users depending on needs
- Actively supporting, developing and changing communication systems for each Service User
- Contributing to the development of a detailed description of how best to communicate with the Service User
- Demonstrating appropriate communication methods at team meetings, and daily interactions with Service Users and colleagues at IGNIS CARE LIMITED

Support Choice:

- Provide experiences that enable the Service User to be able to make an informed choice in respect of activities
- Present opportunities for the Service User to make meaningful choices
- Teach choosing skills

Support Physical and Mental Health:

- Implement individual health Care Plans
- Support Service Users to maintain physical health and wellbeing (cleaning teeth, healthy eating, exercise)
- Support access to health care systems, e.g. visiting GP
- Identify and interpret the Service User's physical and emotional state from non-verbal behaviours (i.e. facial expression, body movements, other behaviour)
- Correctly administer medication according to the agreed protocol
- Ask Mrs Miriam Chengeto Cummings for support/advice when needed
- Articulate what the Service User might feel and think in response to the strategies that are being implemented

Support Relationships with Family, Friends and the Wider Community:

- Actively engage with professionals and family and friends
- Actively support friendships and relationships with others
- Communicate effectively with the Service User's circle of support, supporting then to maintain key relationships, facilitating contact, visits for example and keeping family members and friends informed
- Use formal and informal ways of sharing information
- Seek advice from the circle of support regarding best interest decisions

Support Safe, Consistent and Predicable Environment:

- Use strategies to help the Service Users predict, understand and control their environment (e.g. visual timetable or social stories)
- Identify and avoid, if possible, aspects of the environment that may be a risk factor for a behaviour
- Implement interventions designed to help the Service User cope with challenging environments

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- Develop personal activity schedules with routinely occurring activities as anchors and a menu of other activities for choice and responsive flexibility

Support Appropriate Levels of Participation in Meaningful Activity:

- Identify activities the Service User likes and create opportunities to make them a part of daily life
- Help the Service User do something they like for most of the time
- Help the Service User do things they do not like, but that are essential
- Introduce new activities so the Service User has more activities to choose from
- Support the Service User to develop skills in order to do things as independently as possible
- View complex activities as a series of simpler activities arranged in a sequence of steps that the Service User is able to do with help; adapt the level of help for each step so they can join in as much as possible
- Supply extra motivation and reward for low or non-preferred activities
- Schedule the day so the Service User has at least one activity available at all times, (most often more than one), and the support required to perform the activity
- Intersperse low-preference and high-demand activities with low-demand, high-preference activities
- Keep track of what Service Users do to make sure it is often enough, of good quality, spread out in time, and has enough variety and interest

Knowing and Understanding Relevant Legislation:

- Identify and apply key points from relevant legislation
- Participate in assessing mental capacity of the Service User in everyday Support giving and interactions

A Commitment to Behavioural Skills Training:

- Participate in training programmes identified for all staff at IGNIS CARE LIMITED
- Participate in specific training in the implementation of interventions or support that have been identified within a Behaviour Support Plan

5.11 Reducing Restrictive Interventions

PBS aims to reduce the need for restrictive interventions. In some small circumstances, and in line with the Mental Capacity Act, some Service Users may be subject to restrictive practices including restraint and physical Interventions.

Restrictive interventions must never be used to punish or be used for the purpose of inflicting pain, suffering or humiliation. They must always be proportionate to the risk of harm and the seriousness of that risk and restrictive intervention must represent the least restrictive means by which to meet a Service User's need.

IGNIS CARE LIMITED will ensure that all staff have an understanding of PBS to reduce the need for restrictive interventions which can increase Service User's behaviours.

Restrictive practices, restraint/physical intervention should only be used when an approved, accredited trainer has delivered the training. An approved list can be found via the Certified organisations - Bild - Association of Certified Training (bildact.org.uk)

Before any staff members use restrictive practices including restraint / physical intervention (other than short-term restriction of the Service User's freedom of movement, or in an unforeseen emergency) they will receive appropriate levels of training in line with the Service User's needs, staff roles and responsibilities and will be assessed as competent to carry out any prescribed techniques.

This policy does not cover restrictive practices including restraint and physical intervention. Please read in conjunction the Restrictive Practices including Restraint and Physical Interventions Policy and Procedure at IGNIS CARE LIMITED.

5.12 Post-Incident Action

Following an incident, staff must:

- Notify the GP and other members of the multidisciplinary team if appropriate
- Involve fellow Support Workers in discussions about the cause, learning and future management for the Service User, as per the post-incident review
- Inform their relative or significant other
- Complete the appropriate incident or accident forms
- Complete a regulatory notification, where required (for example police involvement or harm to a Service User)
- Update associated risk assessments and Care Plans
- Introduce a behaviour assessment form, if this is the first episode, or update an existing form accordingly
- Ensure that handover communications include the incident to inform all relevant Support Workers who support the Service User
- Debrief and support any Service Users and Support staff involved who may be emotionally affected by the incident

5.13 Post-Incident Review

A post-incident review with staff must take place as soon as possible and within at least 72 hours of an incident ending. If possible, someone not directly involved in the incident will lead the review. The review will cover:

- What happened during the incident
- Any trigger factors
- Each staff member's role in the incident
- Their feelings at the time of the incident, at the review and how they may feel in the near future
- What can be done to address their concern

A documented record of this must be drawn up, especially in relation to any restrictive or physical interventions used.

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If any Service User is restrained, this must be discussed and the reasons why this was considered necessary must be shared during the review.

Any lessons learned must be discussed with the team and include any relevant multidisciplinary professionals.

5.14 Governance and Corporate Responsibility

IGNIS CARE LIMITED will ensure that robust governance, monitoring and regular reviews are in place where Service Users who are exposed to restrictive interventions have access to high-quality behaviour support plans, designed, implemented and reviewed by staff with the necessary skills, and that restrictive interventions are carried out lawfully. IGNIS CARE LIMITED will ensure that its effective governance frameworks are based on transparency and accountability.

IGNIS CARE LIMITED will ensure that there is an identified director or equivalent who takes a lead responsibility for restrictive intervention reduction programmes and that PBS is embedded into the organisation.

5.15 Learning and Development

Positive Behaviour Support

All key staff will receive relevant Positive Behaviour Support training which will include:

- Why do behaviours happen?
- 10 things you can do for someone displaying challenging, concerning, stressed and distressed behaviours
- Active support
- How to communicate effectively
- Building a good rapport
- The impact of the environment
- Good physical health

All PBS training must be delivered by an accredited training provider.

Oliver McGowan Training

The Health and Care Act 2022 introduced a requirement that regulated service providers supporting Service Users with LD/Autism must ensure their staff receive training on learning disability and autism which is appropriate to the person's role.

IGNIS CARE LIMITED is committed to meeting the legal duty under the Health and Care Act 2022 and the Oliver McGowan Code of Practice (finalised 6 September 2025).

- **Mandatory training**
  - All IGNIS CARE LIMITED staff will complete learning disability and autism training that meets the standards in the Code
- **Role-appropriate**
- This training will match each staff member's role, from awareness to advanced care and support
- **Positive Behaviour Support**
  - IGNIS CARE LIMITED will use training to promote person-centred care, reduce restrictive practices, and support dignity, choice, and independence
- **Compliance**
  - IGNIS CARE LIMITED will maintain records of training, and will ensure staff apply learning in practice

Additional Training

Any additional training will be delivered in line with specific risks or Service User needs. This may include restrictive interventions.

Mrs Miriam Chengeto Cummings will also consider the benefits of additional training which supports a holistic approach to supporting Service Users with challenging, concerning, stressed and distressed behaviours. This may include:

- All About Ourselves and Our Lives: Valuing people with learning disabilities and autism
- Good Communication: Communicating well with people who have learning disabilities and autism
- Growing Older: Providing care for people with learning disabilities who are ageing or at the end of life
- Healthy Sex and Relationships: Supporting people with learning disabilities and autism to have sex and relationships
- Staying Healthy and Well: Supporting good health and wellbeing for people with learning disabilities and autism
- Valuing Autistic People: Valuing difference
- Dementia
- Mental Health conditions
- Brain Injury

Additional information is available in the Further Reading section of this policy.

5.16 Monitoring and Audit

Mrs Miriam Chengeto Cummings is responsible for ensuring the ongoing effectiveness, quality, and safety of Positive Behaviour Support (PBS) practice. Monitoring and audit arrangements are designed to ensure that behaviour support is person-centred, preventative, and aligned with safeguarding and risk management requirements.

Monitoring activities include:

- Regular reviews of Service Users PBS plans and risk assessments to ensure strategies remain effective, proportionate, and responsive to any changes in risk
- Observations of practice and review of records, including staff activity that demonstrates active support and the consistent implementation of PBS strategies

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- Reviewing support provided to Service Users to ensure opportunities are in place to develop new skills, particularly those that improve communication, independence, and overall quality of life
- Assessing staff understanding of preventative strategies, including environmental adaptations, skills development, and the use of individually designed support to reduce the likelihood of behaviours of concern
- Monitoring the effective use of reactive strategies, such as distraction, environmental changes, and de-escalation, with a clear focus on the minimum use of restrictive interventions
- Ensuring appropriate debriefing takes place following any incident, including reflective discussions with staff and support for Service Users, to identify learning and reduce future risk
- Incident, safeguarding, and trend analysis, to identify patterns, emerging risks, or areas requiring improvement
- Audits of Service Users records, including behaviour support documentation, incident reports, safeguarding records, staff training, supervision, and competency assessments
- Discussions with staff to assess their understanding of PBS, its impact on Service Users, and to confirm they receive regular supervision with opportunities to reflect on practice
- Staff feedback, complaints, and concerns, which are used as a source of learning and quality improvement in line with the Complaints, Suggestions and Compliments Policy and Procedure

Where monitoring identifies increased risks, ineffective strategies, or emerging safeguarding concerns, PBS plans, risk assessments, and control measures are reviewed and updated promptly. Learning from audits, incidents, complaints, and safeguarding outcomes is used to improve practice and strengthen risk management arrangements.

5.17 Care and Support at IGNIS CARE LIMITED

IGNIS CARE LIMITED provide care and support to Service User's with a range of needs in a person-centred, safe, and lawful way.

All staff must follow the guidance within this policy and the Service User's Care Plan, ensuring that assessed needs, reasonable adjustments and individual preferences are met.

- Every Service User is treated equally and with dignity and respect
- Care and support are tailored to individual needs, preferences and desired outcomes
- Staff follow legal, regulatory, and professional guidance at all times
- Person-centred approaches are used to promote independence, choice, and wellbeing

To support this approach, staff will also follow the policies and procedures below where applicable:

- Person-Centred Care and Supporting Planning
- Safeguarding Adults
- Raising Concerns, Freedom to Speak Up and Whistleblowing
- Mental Capacity Act (MCA) 2005
- Deprivation Of Liberty in Community Settings
- Consent to Care, Support and Treatment
- Equality, Diversity and Human Rights
- Overarching Medicines Management
- Positive Behaviour Support Including Challenging Behaviour
- Restrictive Practices Including Restraint and Physical Interventions
- Sex, Sexuality and Relationships

This list is not exhaustive and there will be additional policies and procedures in place to support specific Service User needs. Staff must seek clarification from their line manager or the Registered Manager, Mrs Miriam Chengeto Cummings, if there is any uncertainty.

Staff supporting any specialist area of need will receive appropriate induction and training. They will complete competency assessments, where required, to meet the needs of Service User's as outlined in the Training Policy and Procedure at IGNIS CARE LIMITED.

To ensure Service User's receive person-centred care and support from IGNIS CARE LIMITED, staff must adhere to the following :

6. Definitions

6.1 Positive Behaviour Support

- An understanding of the behaviour of a Service User. It is based on an assessment of the social and physical environment, and the broader context in which the behaviour happens, includes the views of the Service User and everyone involved, and uses this understanding to develop support that improves the quality of life for the Service User and others who are involved with them

6.2 Assessment

- The collection and analysis of data relating to the behaviour concerned and includes what happened before, during and after the behaviour. Intensity and duration must also be taken into consideration

6.3 Positive Behaviour Support (PBS) Care Plans

- Must include strategies to reduce the Service User's unwanted behaviours, and enhance their quality of life, their safety, the safety of others and their wellbeing

6.4 Primary Strategies

- Used to reduce the likelihood of the behaviour happening, for example, managing or avoiding "triggers". Primary strategies also provide opportunities for new experiences and acquiring new skills

6.5 Secondary Strategies

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- Used if or when the primary strategies do not work, and there is an escalation of the behaviour — the use of calming approaches, changing the environment or diversion techniques

6.6 Reactive Strategies
- Planned, rigorous interventions that are in place in response to an incident; they aim to take control of the situation and minimise risk

6.7 Reviews
- Must include the support being provided, to ensure that it is effective and reflects current and/or changing needs

6.8 Behavioural Expression Of Emotional Need
- These are behaviours that are outside of the 'social norm' that are communicating an emotion, possibly in the absence of verbal expression, and may be difficult for the person to resolve or is difficult for others to understand.
- Examples of Emotional Need:
  - Comfort
  - Belonging
  - Safety
  - Identity
  - Occupation
  - Sexual
- Examples Of Behavioural Expression:
  - Agitation
  - Aggression (verbal or physical)
  - Pacing
  - Withdrawal
  - Public display of sexual expression

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7. Key Facts - Professionals

Professionals providing this service should be aware of the following:
- The input of other health and/or social care professionals may be needed for the assessment and preparation of the PBS Care Plan
- This policy gives an outline of the standards of practice expected when using PBS, further training, education, and support will be required
- Where behaviours pose a risk to the individual or others, a risk assessment must be carried out

8. Key Facts - People Affected by The Service

People affected by this service should be aware of the following:
- You will be included in the assessment and development of your PBS Care Plan
- You can expect that your family, carer, advocate, or guardian will be involved in the development of your PBS Care Plan
- You will be offered opportunities to acquire new skills to assist you

Further Reading

NHS Health Education England - The Oliver McGowan Mandatory Training on Learning Disability and Autism:

https://www.hee.nhs.uk/our-work/learning-disability/current-projects/oliver-mcgowan-mandatory-training-learning-disability-autism

Skills for Care - Supporting people with challenging or distressed behaviour:

https://www.skillsforcare.org.uk/Developing-your-workforce/Care-topics/Supporting-people-with-challenging-or-distressed-behaviour/Supporting-people-with-challenging-or-distressed-behaviour.aspx

The Challenging Behaviour Foundation:

https://www.challengingbehaviour.org.uk/

BILD Resources including Positive Behaviour Support:

https://www.bild.org.uk/resources/

Positive Behaviour Support resources:

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https://positivebehaviour.org.uk/about-positive-behaviour-support/

BILD Learning and Development - Our workforce development programmes:

https://www.bild.org.uk/workplacedevelopment/

PainChek - A digital pain assessment tool that uses AI to detect to facial expressions of pain:

https://www.painchek.com/uk/case-studies/

NHSE - Identifying restrictive practice weblink:

https://www.england.nhs.uk/long-read/identifying-restrictive-practice/

Outstanding Practice

To be "outstanding" in this policy area you could provide evidence that:

- Care Plans that support concerning, stressed and distressed behaviours show a proactive approach and follow the principles of PBS. These always involve the Service User
- Staff will record when the support of other health and/or social care professionals have been involved in the assessment, planning and review of a PBS Care Plan
- Records showing the frequency, intensity and duration of complex behaviour must be kept, and evaluated against the strategies included in the Service User's Care Plan
- There is evidence of active support with a focus on engagement, meaningful activities and occupational roles, if appropriate
- Staff training records will include training, education and support in the development and use of PBS
- PBS Care Plans focus on the communication needs of the Service User
- There is evidence that a good rapport is built between staff and Service Users which reflects individual needs, preferences and wishes
- The wide understanding of the policy is enabled by proactive use of the QCS App

Forms

The following forms are included as part of this policy:

Title of form	When would the form be used?	Created by
Positive Behaviour Audit Form - CC141	To audit Positive Behaviour Care Plans	QCS
Record of Incidents involving challenging, concerning, stressed or distressed behaviors - CC141	To record all incidents	QCS
Assessment of Behavioural Expression of Need	To analyse the function of a behaviour.	QCS
Abbey Pain Scale	To assess pain in a person with communication or sensory needs.	QCS
Stop and Pause - CC141	Support the management of a person living with Dementia	QCS
Risk Factors of Disturbed or Violent Behaviour - A Guide For Staff - CC141	As an information tool for staff awareness	QCS

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Positive Behaviour Audit Form - CC141

Positive Behaviour Care Plan Audit

Service User Name:

Is there evidence to support that the PBS policy and procedure is implemented in practice? (Risk screening is undertaken, evidence of PBS support plans, robust evidence of active support)
How many PBS Care Plans are in place within IGNIS CARE LIMITED?
Do all Service Users who have a PBS Care Plan in place have a PBS assessment in place?
Is there evidence of input/consultation with other health or social care professionals?
Do all PBS Care Plans contain primary, secondary, and reactive strategies?
How often are PBS Care Plans reviewed?
Is there evidence that the Service User, their family, carer, advocate, legal representative or guardian have been involved in the development of the Care Plan?
Is there evidence that the Service User, their family, carer, advocate, legal representative or guardian have been involved in the review of the Care Plan?

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How often are primary strategies effective?
How often are changes, if any, made to primary strategies?

How often are secondary strategies used?
How often are changes, if any, made to secondary strategies?

How often are reactive strategies used?
How often is the reactive strategy used first?
How often are changes, if any, made to reactive strategy?

General comments/notes:

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Date	By	Review date

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Record of Incidents involving challenging, concerning, stressed or distressed behaviors - CC141

Service User's name:		**Date:**
Support Worker or Carer (completing this form):		**Time:**
Location of event:		
Precipitating factors:		
Account of event:		
Account of Injuries:		
Action Taken:	**Solutions** (Tick as appropriate):	
	De-escalation techniques	
	Snoezelen (Controlled Multi-sensory environment) or similar	
	Diversion	
	Specific staff member	
	State type of diversion or name of staff member:	
Follow up summary:		

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Assessment of Behavioural Expression of Need

Behaviour Chart (The ABC of Behaviour)		
Service User's Name:	**Service User Number:**	**Date Chart Started:**

Care staff can refer to the Service User's Care Plan in place to support how potential behaviours can be managed.

Date	Behaviour	Details
		What happened before (Antecedent):
Initials		**What happened during (Behaviour observed):**
		What happened after (Consequences):
		Interventions Necessary:

Date	Behaviour	Details
		What happened before (Antecedent):
Initials		**What happened during (Behaviour observed):**
		What happened after (Consequences):
		Interventions Necessary:

Date	Behaviour	Details
		What happened before (Antecedent):
Initials		**What happened during (Behaviour observed):**
		What happened after (Consequences):
		Interventions Necessary:

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Abbey Pain Scale

For measurement of pain in people with learning disabilities, dementia or those who cannot verbalise.

Name of Service User:		D.O.B	Time last pain relief was given:		
			AM / PM		
Vocalisation (e.g. whimpering, groaning, crying)		Score	Behavioural Change (e.g. increased confusion, refusing to eat, alteration in usual patterns)		Score
Absent	0		Absent	0	
Mild	1		Mild	1	
Moderate	2		Moderate	2	
Severe	3		Severe	3	
Facial Expression (e.g. looking tense, frowning, grimacing, looking frightened)		Score	Physiological Change (e.g. temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor)		Score
Absent	0		Absent	0	
Mild	1		Mild	1	
Moderate	2		Moderate	2	
Severe	3		Severe	3	
Change in body Language (e.g. fidgeting, rocking, guarding part of the body, withdrawn)		Score	Physical Changes (Skin tears, pressure areas, arthritis, contractures, previous injuries)		Score
Absent	0		Absent	0	
Mild	1		Mild	1	
Moderate	2		Moderate	2	
Severe	3		Severe	3	

Total Score: Tick box for Total pain score	0-2 No Pain	3-7 Mild Pain	8-13 Moderate Pain	14+ Severe Pain

Type of Pain	Chronic	Acute	Acute on Chronic

Signature of Staff:	
Date:	

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Pain Scale – Review

Name of Service User:	
Date of Birth:	

Year:	Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sept	Oct	Nov	Dec
Vocalisation												
Facial Expression												
Change in body language												
Behavioural Change												
Physiological Change												
Physical Changes												
Score												
Change in Care Plans Y/N?												
Staff Signature												

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Stop and Pause - CC141

Helping someone with dementia who is distressed or behaving unusually

There are often good reasons why someone with dementia is distressed or behaving unusually. The person's brain is working differently and can affect their normal character and many things on a daily basis. This may include forgetting things, repeating things over and over and misunderstanding what others say. The person might not always be able to tell you what's troubling them. Sometimes we react to unusual behaviour without knowing how the brain changes have affected the person's abilities or what they might need or be saying through their behaviour. It can be difficult to work out what the likely cause is and what you can do to help, for the benefit of both of you.

The Alzheimer's Society has more advice and information for people with different types of dementia, not just Alzheimer's dementia. You can go to the website www.alzheimers.org.uk or call them on their helpline (Freephone) on 0300 222 11 22. We have listed useful factsheets on particular behaviours at the end of this leaflet.

If you would like further help, please don't struggle on your own as there are many ways in which people in your situation can be helped. Please contact your local Alzheimer's Society or your doctor for advice. If you want information on how services can help you, see *How Health and Social Care Professionals Can Help*. http://alzheimers.org.uk/factsheet/454

We know that you have probably tried many things and there is often no 'right way' or magic solution. Some approaches work better for some people and situations than others. Also, they may work at one time and not another. It can help to see yourself as exploring what the most helpful approaches are. We have used the ideas of "STOP" and "PAUSE" to describe the key ways to help you listen and watch, to improve your understanding of distress and unusual behaviour.

You may need to try some things for several weeks before you see improvement. If distress or behaviours do not resolve with the advice given below, consult your doctor or ask for a referral to your local specialist mental health service, which works with people with dementia and their families.

You respond… 'STOP' and 'PAUSE'

STOP

S – **See** things from the point of view of person with dementia

T – **Think** about your own thoughts and feelings

O – **Observe** and ask what the person is trying to communicate and what is going on

P – **Patience** and persistence

PAUSE

P is for PHYSICAL

Are they in pain?

Pain can be a common cause of changes in behaviour and can result from many problems such as joint pain, dental problems or discomfort from skin problems or constipation.

What to do: Ask the person if they are in pain. Watch out for signs of them being in pain. Change their position if they have been sitting in one place for a long time. If you think they are in pain, get advice from their doctor. Note the activity they're doing when they are distressed or seem uncomfortable so you can give information to their doctor. Please seek advice from their doctor if the person is taking any medication for pain, either prescribed or purchased. It may be that prescribed painkillers need adjusting or that those being self-medicated are unsuitable.

See the factsheet '*Pain in dementia'* from the North West Dementia Centre:
https://www.pssru.ac.uk/pub/MCpdfs/Pain_factsheet.pdf

Has their medication been reviewed or changed recently? Are they taking all their medication correctly?

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New medicines might be causing side-effects. Older medicines may no longer be needed or may need adjusting. Incorrect use of medication may result in extra side-effects or result in limited or no expected benefits.

Do they have an infection?

They might have an infection such as urinary tract or chest infection or cellulitis. This can lead to changes in behaviour, such as confusion.

What to do? Look out for things like smelly or cloudy urine (wee) or an unusually wheezy chest or redness, itchiness or soreness of the vagina (women) or penis (men), or red and hot patches of skin and report these to their GP.

Are they hungry or thirsty?

Dementia can cause changes in taste and appetite. People may have difficulty managing or recognising food or cutlery. Dehydration (lack of liquid) can happen with changes between hot and cold weather. People may avoid drinking in order to avoid going to the toilet. Being dehydrated can lead to further problems.

What to do: Note any problems with eating or drinking. People may need prompting to use cutlery, such as putting a fork in their hand and guiding their hand to the food. Meals may need to be small and often to ensure that blood sugar is maintained. Look for very yellow urine, which is a sign that they ought to be drinking more. Encourage drinking and provide support for going to the toilet if needed (see advice in Self-esteem below). Look for problems with denture pain or mouth ulcers. Let their GP know if you are concerned about how much they are eating or drinking.

See factsheet *Eating and drinking* on the website: http://alzheimers.org.uk/factsheet/511

Are they getting enough sleep at night?

Dementia can cause changes in people's sleep schedule so that they wake up more often and stay awake for longer at night. Confusion about time can lead them to think it is daytime at 4am and want to get dressed.

What to do: Note any signs of pain or discomfort upon waking. Keep bedtime routines and provide nightlights and comfort objects. Avoid watching TV in the bedroom or the person spending long periods of time in bed while awake. Use bed for sleep and sex. Encourage outdoor exercise or activities to keep them alert during the day. Try to stop or reduce daytime napping. Avoid alcohol and caffeine before bedtime. See their GP if problems persist.

Could they have hearing or eyesight problems?

People can become disinterested in a conversation or an activity just because they cannot see or hear easily.

What to do: Check how well they can see or hear things, even if they have glasses or a hearing aid. Improve the lighting. Make sure that you talk loudly and clearly into the good ear. Avoid competing noises or activities such as TV or radio. Try to move slowly and approach the person from the side where the eyesight and/or hearing are best. Get advice from an optician or hearing specialist if you think their sight or hearing could be improved.

Could they be making 'visual mistakes'?

People with dementia might still have good vision but have problems with making sense of things correctly in front of them (called visuospatial difficulties). This might make it difficult for them to watch TV, use objects correctly or walk confidently. Other examples include misinterpreting reflections in mirrors or avoiding stepping on shiny floor because it looks wet or slippery.

What to do: Improve the lighting. Make sure the rooms are free from clutter and there is space to move around with confidence. Cover-up or change busy patterns on walls and floors. Help the person recognise objects. Do this by showing them how to use the object, getting them to touch the object or using noise, e.g. flushing toilet. Use short simple statements rather than questions or gestures to indicate walking to the toilet, etc. For example, say "come to the toilet" rather than "would you like to go to the toilet"?

Could they be experiencing hallucinations?

Hallucinations may occur with some types of dementia, especially dementia with Lewy bodies. Visual hallucinations are most common and involve seeing things that are not present, usually people and animals. This can be frightening and lead to changes in

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behaviour.

What to do: If they are not worried then don't dwell on it. Listen carefully and acknowledge what the person is saying. Talk calmly and try not to argue with them. Consult their GP if the hallucinations persist or worsen or are frightening.

Could the room temperature be too hot or too cold?

What to do: If it is very hot and the temperature cannot be reduced, consider giving them more drinks, use fans or sit them outside in the shade. If it is cold, try the use of blankets and extra clothing.

A is for ACTIVITIES

Could they be bored or needing social contact?

What to do: Use simple activities to prompt conversation, such as looking at a vase of flowers, a picture on the wall or looking out of the window. Involve them in everyday activities like laying the table. Try and do activities they used to enjoy doing, e.g. gardening or visiting the seaside. Give the person regular opportunities to talk to someone. If one is near, visit your local dementia café where both of you can meet and chat with others in a similar situation (contact your local Alzheimer's Society for more information). See factsheet on 'Keeping active and involved' at: https://www.alzheimers.org.uk/get-support/publications-and-factsheets/living-with-dementia-active-and-involved

Is there too much going on or is the person in unfamiliar surroundings with people they don't recognise?

What to do: Consider having more routine and structure in the day by doing the same things at the same time every day; have a quiet-time or use calming activity or music, especially at times they are tired, such as after lunch.

U is for YOU

Are you looking after yourself?

Your situation may be extremely difficult to cope with and you may feel helpless and frustrated. It is important that you look after yourself and your health and have support. You are not going to get it right all the time. It's important that you do not take all the responsibility for managing very demanding situations.

What to do: Try to share the responsibilities with others and accept help from family, friends, neighbours or professionals. Many people benefit from talking with people in a similar situation. You can find information and support from your local Alzheimer's Society branch or carers support organisations (contact details at bottom). You can get ideas on how to solve problems or plan for future living arrangements.

See factsheet *'Carers: Looking after yourself'* at: http://alzheimers.org.uk/factsheet/523

If you are providing support for someone who cannot manage without it, you are legally entitled to a Carer's Assessment under the Care Act. This assessment is about your needs and what support you need in your caring role. To seek a Carer's Assessment you can call your local Social Care Services or ask someone from your local Carers Centre.

How do you manage the effects on your relationship?

Dementia has probably had an enormous impact on your relationship, both in practical and emotional ways. It is normal to want to want to turn the clock back to how things used to be before the dementia. You may experience changes in your usual roles, talking and sharing together and closeness. You may find some social situations difficult and embarrassing. Some friends may also avoid you. All of these changes can be experienced as a painful loss.

Within your relationship, you may see things differently from each other. One of you is aware of the difficulties and the other may be unaware or does not seem concerned. Dementia can cause people to forget or be unaware of their difficulties as well as how their actions affect others. This is due to brain damage and is not done on purpose. You might have different ways of coping with the dementia, for example, one of you might downplay the difficulties and one of you might be more accepting and open in talking about dementia.

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What to do: Try to continue with the important things in your relationship, including contact with others. Accept that you might need to take responsibility for looking after both of you.

See the changes as the result of the dementia symptoms and the different ways in which people cope and notice the moments when you are sharing or enjoying things together. Talk about the impact of the loss and changes if you want to. If you need time on your own or with others, having time apart might improve your mood and relationship.

If you want to feel closer together, do things that you can both do, for example, life story work. Develop a “life story” together to support reminiscing and conversations.

Do you understand why they are distressed or behaving out of character for them?

You may struggle to understand someone’s changing behaviour. How you understand the behaviour is crucial to how you will react. If you blame yourself or the person, you are more likely to get angry or frustrated.

What to do: Try and find out as much as you can about dementia and what causes certain behaviours. Try to avoid taking things personally or having arguments over mistaken ideas or attempt to change their viewpoint. Your arguments will only end up frustrating you and probably upsetting them. Be mindful of your own tone and facial expressions and try to speak calmly.

S is for SELF ESTEEM

Are they frustrated because they are unable to communicate their needs or they can no longer do the things they used to do?

People with dementia can find it difficult to feel good about themselves. They may struggle to adjust to the effects of dementia because they cannot maintain the same skills and activities. This can often be expressed through mood changes or unusual behaviours. You may be tempted to do things for the person to help them and to make life easier. This is understandable. However, your intention to make life easier could result in taking away the person’s confidence and independence in doing things themselves. The saying, ‘if you don’t use it, you lose it’, can be true.

What to do: The aims of the following ideas are to support the person with dementia to adjust to the effects of the dementia, to live life as independently as possible and to engage in social and meaningful activities. Include people in conversations and be aware of how they might be feeling. Let the person finish their sentences unless they ask for your help. Don’t point out their mistakes. Keep the flow of conversation going – move on if the person has trouble finding a word or appears anxious. Let them do jobs they are used to doing, e.g. putting some of the shopping away. Break the job down into smaller steps to help them. This will help them feel they are doing something useful. Explain what you plan to do or what you are doing. Ask them questions which require 'yes'/'no' responses and give plenty of time to respond.

See factsheet: *Communicating at* https://www.alzheimers.org.uk/sites/default/files/2020-03/communicating_500.pdf

E is for EMOTIONS

Are they sad, scared, depressed or anxious?

People with dementia still experience feelings and emotions even though they may not be able to explain to you their feelings or remember what caused them to feel that way.

What to do: Note down what was going on to see if something triggers the change in feelings or mood. This might be due to certain music, noises or a visit from someone. Encourage distracting activities such as walking. Touching or holding their hand may help calm them and show them you care. Try to pick out key words or phrases and repeat these back as it may help the person focus on a particular topic. Respond to the person’s feelings rather than correcting the accuracy of what they are saying. For example, if someone says they miss their mother, think about the meaning behind what they are saying. Are they sad or worried about something? You could encourage them to tell stories about their mother and what they miss about her to help them feel more secure. You might need to try out different ways of responding to see what works best.

If someone’s low or anxious feelings or mood persists, ask their doctor for a referral to specialist mental health services.

Other useful factsheets from Alzheimer’s Society:

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Unusual behaviour - https://www.alzheimers.org.uk/about-dementia/symptoms-and-diagnosis/symptoms/behaviour-changes

Coping with incontinence - https://www.alzheimers.org.uk/sites/default/files/2018-10/factsheet_continence_and_using_the_toilet.pdf

Dressing - http://alzheimers.org.uk/factsheet/510

How does dementia affect sex and intimacy?- https://www.alzheimers.org.uk/get-support/daily-living/sex-intimacy-dementia

Moving and walking about - http://alzheimers.org.uk/factsheet/501

Washing and bathing - *https://www.alzheimers.org.uk/get-support/daily-living/washing-dressing*

Dealing with aggressive behaviour - http://alzheimers.org.uk/factsheet/509

Visuoperceptual difficulties - http://www.alzheimers.org.uk/factsheet/527

Hallucinations- https://www.alzheimers.org.uk/about-dementia/symptoms-and-diagnosis/hallucinations

Local Authorities

The relevant Local Authority varies depending on the service user's home location. The correct Local Authority is recorded in the service user's Care Plan and PASS profile.

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Risk Factors of Disturbed or Violent Behaviour - A Guide For Staff - CC141

The following lists are not intended to be exhaustive and these risk factors should be considered on an individual basis. Those factors will include factors linked with demographic or personal history and should be taken into account when assessing the risk of disturbed/violent behaviour, including the following features:

- History of disturbed/violent behaviour
- History of misuse of substances or alcohol
- Support worker reporting the Service User's previous anger or violent feelings
- Previous expression of intent to harm others
- Evidence of rootlessness or 'social restlessness'
- Previous use of weapons
- Previous dangerous impulsive acts
- Denial of previous established dangerous acts
- Severity of previous acts
- Known personal trigger factors
- Verbal threat of violence

Evidence of recent severe stress, particularly a loss event or the threat of loss. These factors should be considered in combination with any of the following:

- Cruelty to animals
- Reckless driving
- History of bed-wetting
- Loss of a parent before the age of eight years

In addition to these factors, clinical variables should be taken into account when assessing the risk of disturbed/violent behaviour, including the following features:

- Misuse of substances and/or alcohol
- Drug effects (disinhibition, akathisia)
    - Active symptoms of schizophrenia or mania, in particular:
    - Delusions or hallucinations focused on a particular person
    - Command hallucinations
    - Preoccupation with violent fantasy
    - Delusions of control (especially with a violent theme)
    - Agitation, excitement, overt hostility or suspiciousness
    - Poor collaboration with suggested treatments
    - Antisocial, explosive or impulsive personality traits or disorder
    - Organic dysfunction

In addition to these factors, situational variables should be taken into account when assessing the risk of disturbed/violent behaviour, including the following features:

- Extent of social support
- Immediate availability of a potential weapon
- Relationship to the potential victim (for example, difficulties in relationship are known)
- Access to the potential victim
- Limit setting (for example, staff members setting parameters for activities and choices.)
- Staff attitudes

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